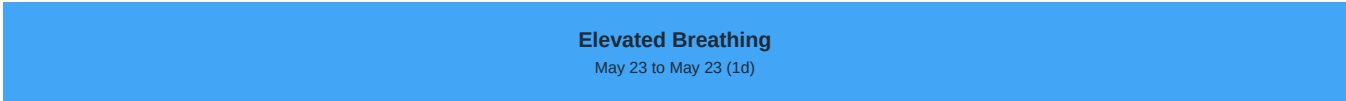


Patient ID: TaylorPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 743/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01

■ HR ■ Breathing



Last 24 Hours

Vital		Avg	Min	Max
Heart Rate		63 bpm	51 bpm	99 bpm
Breathing		16 brpm	12 brpm	30 brpm
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
1	1h	Elevated Breathing	<1	100%

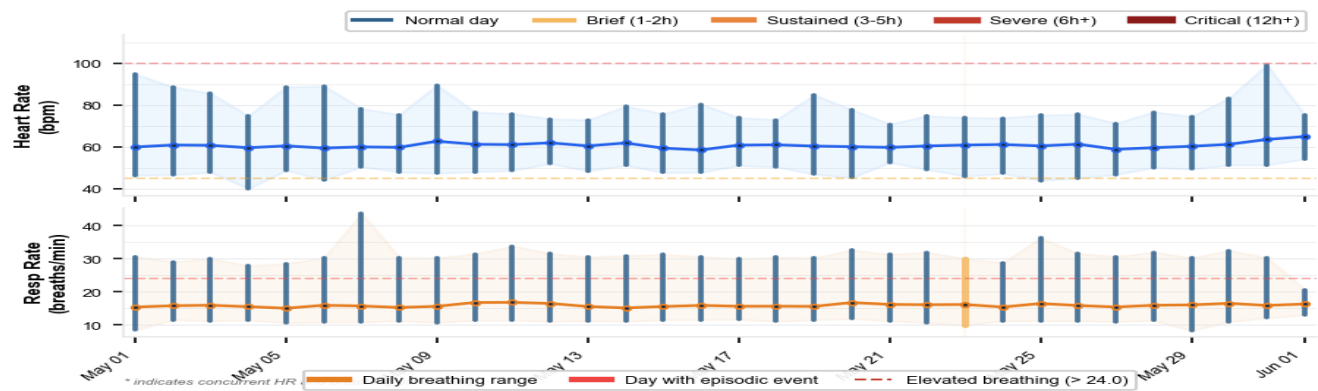
Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 23	8 AM to 9 AM	1h	1	Elevated Breathing	25	14	30	Check SpO2 and lung sounds; assess for fluid overload or early infection

Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

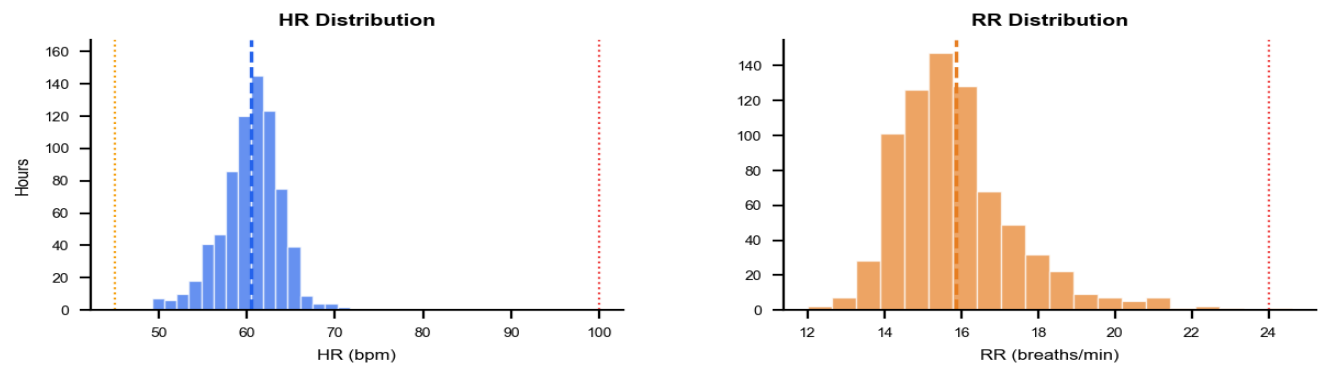
Suggested Clinical Review Actions

- Elevated Breathing (avg 25 brpm, peak 30 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

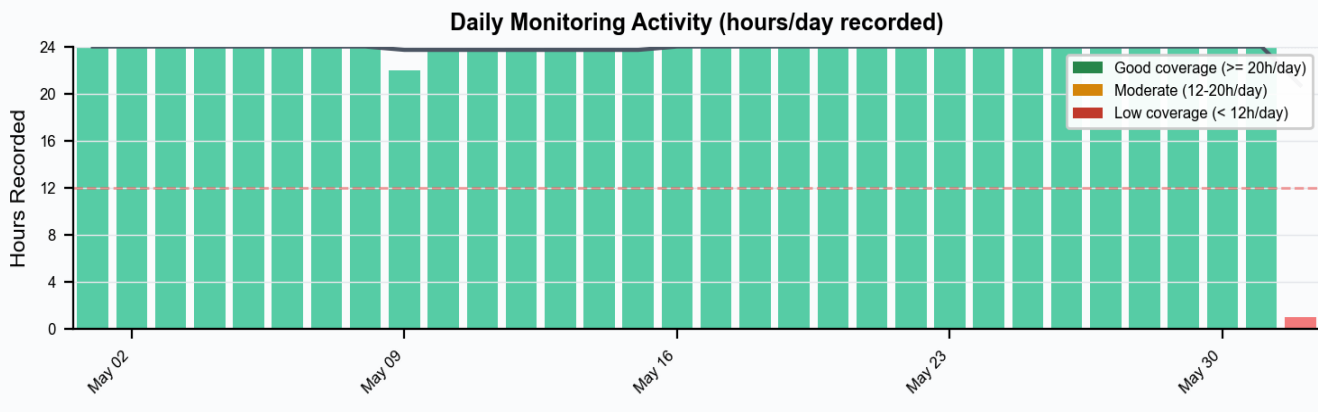


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode, RR episode, Recorded, no episode, No data

Clinical Alerting Thresholds

Clinical Alerting Thresholds:

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 bpm
- High Breathing > 30 bpm
- Very High Breathing > 40 bpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.